**BUSINESS CASE** OTTAWA COMPANION TO THE PHASE 0 SURVEY AUGUST 2026

# Ottawa service business case

A case for **operating** a non-urgent patient transportation service in Ottawa. It reaches roughly **$1.95M revenue and $242K EBITDA in year three** on 13 vehicles, for about **$850K of funding** — and whether that is a good business turns on a single contract.

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## 01 Read this first

**This contradicts the survey's own recommendation, deliberately.** The Phase 0 survey concludes that the value in Ontario is a coordination and settlement layer, and that running vehicles is a low-margin, labour-constrained business. This case was requested and is delivered in full — §07 states plainly where the two conflict.

### Figures are modelled, not quoted

Cost inputs come from published market rates and wage surveys — not vendor quotes, not a broker, not a signed rate card. **Six of nine load-bearing inputs are modelled rather than sourced.** This is a structure for thinking, not a forecast.

**Do not take this to a lender or an investor** until the input register's estimates are replaced with real quotes.

## 02 Unit economics

One wheelchair-accessible vehicle, single operator, at mature utilisation. The design decision that carries the model is **pooling dialysis runs** — carrying two or three wheelchair patients per trip to the same unit is what moves a vehicle from ~5 billable legs a day to ~8.

The base case is **dialysis-anchored and prices every leg at the $85 contract rate**. Ad-hoc discharge work bills ~$150 a trip and would earn ~10% more, but it is unscheduled and unpoolable, so it is treated as upside rather than blended in.

> **Corrected in review.** An earlier draft quoted a "$120 blended rate" alongside the $150,000 figure. The two were inconsistent — $120 across 8 legs a day gives $212,160 — which made the trip mix unauditable. The blended rate is gone; the base case now prices one way.

PER VEHICLE, PER YEAR

### At 85% utilisation, 260 operating days

| ITEM | MODELLED | BASIS |
|---|---|---|
| **Revenue** | **$150,280** | 8 pooled dialysis legs/day × $85 × 260 days × 85% |
| Operator wage + burden | $61,200 | $21/hr + ~22% burden + 15% relief coverage |
| Vehicle lease | $19,200 | ~$1,600/month |
| Fuel | $8,680 | 40,000 km @ 14 L/100km @ $1.55/L |
| Maintenance and tires | $6,000 | Modelled |
| Insurance | $5,500 | Broker quote required — likeliest input to be wrong |
| Licensing, permits | $1,500 | Modelled |
| **Total direct** | **$102,163** | |
| **Contribution** | **$47,837** | ~32% gross margin |

STRETCHER TIER — A DIFFERENT BUSINESS

## Modelled separately · not included in the three-year projection

| ITEM | WHEELCHAIR VAN | STRETCHER VAN |
|---|---|---|
| Crew | 1 operator | **2 attendants** |
| Trips per day | 8 pooled legs | 4 — cannot be pooled |
| Rate | $85/leg | ~$350/trip |
| **Revenue** | $150,280 | $309,400 |
| Crew cost | $61,200 | **$122,400** |
| Lease, fuel, maintenance, insurance, licensing | $40,963 | $51,546 |
| **Total direct** | $102,163 | $173,946 |
| **Contribution** | **$48,117** | **$135,454** |

A stretcher vehicle contributes ~2.8× a wheelchair van despite double the labour, because it bills roughly 2× and cannot be undercut by rideshare. The catch is on the demand side: stretcher work is hospital-discharge driven, so it is **ad-hoc and unschedulable** — precisely the utilisation risk the phasing exists to avoid. A strong second phase and a poor first one. All stretcher figures are modelled, not quoted.

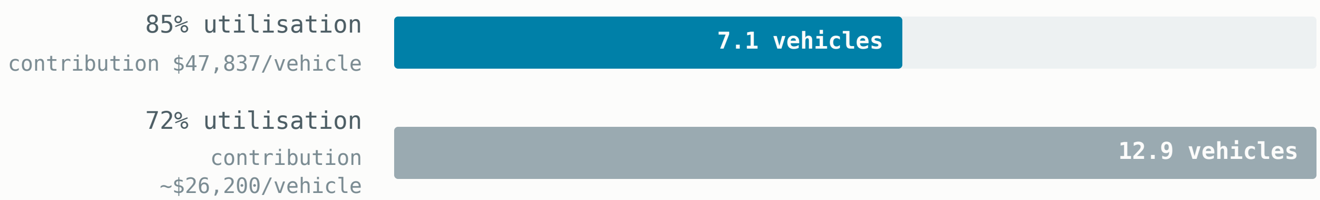
### Direct cost is ~90% fixed per vehicle

The operator is paid whether the van is full or empty. **Utilisation is the entire business.** A van at 55% utilisation loses money; the same van at 85% makes $48K.

## 03 Break-even, and how fragile it is

VEHICLES NEEDED TO COVER $337,500 OF FIXED OVERHEAD

### By utilisation rate




A 13-point drop in utilisation nearly doubles the fleet needed to break even. The sensitivity is not linear, because direct cost barely moves with utilisation. This is the single most important number in the document.

**This is not a two-van side business.** Seven vehicles running near 85% is the floor — which means the contracted volume has to exist *before* the fleet does.

### Lease, don't buy

Purchase is ~$95,000 per converted vehicle. Buying is about $5,600/vehicle/year cheaper and costs $1.2M of capital across the plan. In a business whose binding risk is filling vehicles rather than owning them, **leasing converts capital risk into a cancellable operating cost.** Revisit once utilisation is proven.

## 04 Three-year projection

| | YEAR 1 | YEAR 2 | YEAR 3 |
|---|---|---|---|
| **Wheelchair vehicles (avg)** | 5 | 9 | 13 |
| Average utilisation | 55% | 72% | 85% |
| **Revenue** | $485,000 | $1,143,500 | $1,950,000 |
| Direct costs | $495,000 | $908,000 | $1,328,000 |
| Contribution | −$10,000 | $235,500 | $622,000 |
| Fixed overhead | $290,000 | $337,500 | $380,000 |
| **EBITDA** | **−$300,000** | **−$102,000** | **+$242,000** |

### This models a wheelchair-only fleet

**Corrected in review.** An earlier draft priced all 13 vehicles at the single-operator rate while the service model proposed stretcher vehicles needing **two** attendants. Four such vehicles would add ~$245,000 of labour — **more than the entire stated year-three EBITDA.**

Stretcher capability is now modelled separately in §02 and is a deliberate later decision, not an assumed part of this plan.

### Year 1 contribution is negative

Five vehicles at 55% utilisation do not cover their own operators, before a dollar of overhead. That is the shape of this business, not a modelling artefact — and it is why the gate in §07 matters.

Profitability arrives in **year 3**, not year 2: nine vehicles at 72% is still short, because break-even at that utilisation is ~13 vehicles.

| $402K | $180K | $95K | $850K |
|---|---|---|---|
| Cumulative operating losses, years 1–2 | Working capital for hospital receivables at 60–90 days | Equipment, software, legal, licensing and incorporation | Total funding requirement, including 15% contingency |

## 05 What has to be true

**01** **Utilisation reaches 85%**

At 70% the mature fleet contributes ~$25K per vehicle and break-even moves from 7 to 13. The plan does not survive sustained low utilisation.

CRITICAL

**02** **Crews can be recruited and kept**

Thirteen vehicles needs ~16 operators at ~$21/hr in a tight Ottawa labour market. **Labour, not capital or demand, is the binding operational constraint** — and turnover directly destroys utilisation.

CRITICAL

03

### Contracted volume precedes fleet growth

Every vehicle added ahead of committed volume costs ~$102K a year whether or not it moves.

HIGH

04

### Receivables get collected

Hospital and LTC contracts pay slowly; patient pay is fast but small. The $180K working-capital line is not padding.

HIGH

05

### Transdev/Voyago does not price to defend

A global operator can run Ottawa below cost far longer than a startup can. Real and unhedgeable.

HIGH

## 06 The arithmetic problem

The Phase 0 survey sizes the *entire contestable private patient-transfer market in Ontario* at **$35–116M a year**. Ottawa is ~6.7% of Ontario's population, implying an Ottawa pool of roughly **$2.3M–$7.8M**.

### A 13-vehicle operation would hold 25%–83% of that pool

Against Transdev/Voyago — who own the incumbent, with goPatient already installed at contract hospitals — and Ontario Patient Transfer, who run an Ottawa super base.

**That share is not winnable by a new entrant competing for the same trips.**

### Which means the business is not what it looks like

The plan only works if it is **not** a fight for existing private-transfer volume. The Ottawa Hospital dialysis book — ~109,000 round trips a year — is largely **not in that pool today**. Those patients get there by family car, Para Transpo, taxi, or they don't get there at all: 10% of dialysis patients miss at least one session a month, with transportation a contributing cause.

So the real proposition is **converting unserved and self-served dialysis travel into contracted service**, not taking share. Capture 25% of that book — 27,250 round trips, 54,500 legs at $85 — and that is **$4.6M of revenue from one contract**, more than the entire three-year plan above.

## 07 The decision

GO / NO-GO GATE

**Do not spend $850,000 until The Ottawa Hospital renal program is engaged**

1. **A signed contract, a funded pilot, or a written expression of interest** from the Regional Nephrology Program. Everything else in this document is secondary to that one relationship.
2. **With it:** the fleet fills from day one, utilisation risk mostly disappears, and the recruitment problem becomes tractable because the schedule is predictable.
3. **Without it:** you are a seventh entrant in a $2–8M market against a global operator. The answer is no.

**One conversation** → Missed treatments and cancelled slots → What transport costs them today → Would a pooled contracted service interest them

### The first step is not fundraising

It is a single conversation with the renal program about missed treatments, what transportation currently costs them in cancelled slots, and whether a contracted pooled service interests them. **That conversation costs nothing and determines everything.**

### Reconciling with the survey

The survey says build a coordination layer, not an operator. Both can be true in sequence — and the operator path has one advantage the platform path lacks: **operating the dialysis book generates the scheduling, utilisation and outcome data a coordination product needs to be credible**, and it puts you inside the buyer relationship the platform would otherwise have to cold-sell.

The hybrid — operate the anchor book, instrument it, then sell the coordination layer outward from a working reference site — is more defensible than either alone. It is also more capital-intensive and slower. **That trade is the decision, and it belongs to whoever is signing.**

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**Input register.** Nine load-bearing inputs; six are modelled rather than sourced. Firmest: dialysis population and market wheelchair rates. Weakest: the $85 contract dialysis leg, the $5,500 insurance line, trips per vehicle per day, and the utilisation ramp — all of which need quotes or a pilot before this projection is relied on. The full register with sources sits in **docs/05-ottawa-business-case.md §8**.

Companion documents: **docs/01-market-survey.md** (§07 Ottawa, §08 Toronto) · **docs/02-opportunity-model.md** · **docs/03-source-register.md**